Lakshmi Memorial Hospital Laboratory

Swaroop Nagar, Kanpur 208002 | Ph: 0512 255 9000

DISCHARGE SUMMARY

Patient Name	**Ramakant Dwivedi**	Accession No.	**LMH/2024/04096**
Hospital No	**LMH/704628**	Sample Collected	**22/01/2024 4:10 AM**
Age / Sex	**65 Y / M**	Report Released	**22/01/2024 7:40 AM**
Referred By	**Dr. Praveen Agarwal, MBBS, MD (Pulmonary Medicine)**	Specimen	**Whole blood (EDTA), Serum, Sputum**

DIAGNOSIS

Community acquired pneumonia, right lower lobe. Chronic obstructive pulmonary disease, moderate. Current smoker.

HISTORY

Admitted through casualty with four days of fever, productive cough and increasing breathlessness. Saturation 88 per cent on room air at presentation. Forty pack-year smoking history, three to four chest infections each winter for the past five years.

EXAMINATION

Febrile at 39.5 C, respiratory rate 28, bronchial breathing at the right base with coarse crackles. No signs of cardiac failure. Chest radiograph confirmed right lower lobe consolidation.

COURSE

Started on intravenous antibiotics and controlled oxygen. Fever settled by the third day and oxygen was weaned by the fourth. Inflammatory markers fell steadily, as below. Counselled at length on smoking cessation and started on nicotine replacement on the ward, which he accepted for the first time.

INVESTIGATIONS

ON ADMISSION (22/01/2024)

INVESTIGATION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Total Leucocyte Count	**21400**	**/uL**	**4000 - 10000**
Neutrophil %	**89**	**%**	**40 - 80**
CRP	**246.0**	**mg/L**	**< 6**
Hb	**13.4**	**g/dL**	**13.0 - 17.0**
Na	**131**	**mmol/L**	**136 - 145**
UREA (SERUM)	**52**	**mg/dL**	**17 - 43**
CREATININE (SERUM)	**1.24**	mg/dL	0.7 - 1.3

MICROBIOLOGY

INVESTIGATION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Sputum Culture	**Streptococcus pneumoniae, moderate growth. Sensitive to Ceftriaxone, Amoxicillin.**		**No growth**
Procalcitonin	**4.62**	**ng/mL**	**< 0.50**

TREATMENT

Injection Ceftriaxone 1 g twice daily for five days, Azithromycin 500 mg daily for five days, nebulised bronchodilators, controlled oxygen, nicotine patches.

ADVICE

Complete the oral antibiotic course. Continue inhalers. Do not resume smoking. Review in the chest clinic in six weeks with a repeat CRP and chest radiograph. Return earlier if fever or breathlessness returns.

Tested by: Y. Gaikwad (DMLT)
Results released electronically. Verified before release.

M C Pal

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